

Set 1 — A History of Mistrust

Slide 1

A History of Mistrust:

Why Minority Communities

Struggle to Trust Doctors

(Note: Alternate title version provided: A History of Mistrust: ##### Why Some Communities Struggle to Trust Doctors)

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Fun Fact:

Being LGBTQ+ is normal and okay

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Un-funfact:

Homosexuality was classified

as a mental disorder that was

treated with harmful

conversion therapy or even

shock therapy until 1973.

Research: APA's removal of homosexuality from DSM-II in 1973; history of conversion therapy practices.

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POC & LGBTQ+ individuals often face

systemic discrimination

when seeking healthcare.

Research: Study or report documenting healthcare discrimination by race/sexual orientation (e.g., KFF, Lambda Legal, AAMC).

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Black, Indigenous, and Latinx women have been

sterilized without consent

well into the 20th century.

Research: Specific documented programs — NC Eugenics Board, IHS sterilizations of Native women (1970s), California prison sterilizations (through 2010), Madrigal v. Quilligan.

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Tuskegee Experiment (1932-1972):

American healthcare providers lied to and denied

Black men treatment for syphilis to study the

progression of the disease,

even after a widespread cure was developed.

Research: CDC/NCBI historical record of the USPHS Syphilis Study.

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The US Government's slow response

disproportionately harmed LGBTQ+ & POC

communities.

Current information on the subject has

been removed from USA.gov.

Research: Document which page(s) were removed and when — Wayback Machine snapshot. Cite what the slow response refers to (AIDS crisis government inaction, Reagan administration).

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Mistrust leads to:

* Late diagnoses

* Poorer mental health

* Non-adherence to necessary treatment

* Higher mortality rates

Research: Disparity outcomes literature linking medical mistrust to these outcomes (e.g., NIH, Health Affairs, JAMA).

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“Health is more than the absence of disease.

Health is about jobs and employment, education, the environment, and all

of those things that go into making us healthy.”

-Dr. Joycelyn Elders

Research: Original source + year for this quote (speech, interview, or publication).

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The first step to rebuilding trust is understanding why

it was broken.

Share this series with your community and

let's start that conversation together.

Set 2 — AIDS Care in Marginalized Communities

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AIDS Care in Marginalized Communities

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Anyone who is sexually active could be at risk for sexually transmitted

diseases or infections.

However, Black & Latinx

individuals are 3x more likely to

contract HIV. LGBTQ+ individuals

are at an even higher risk.

Why?

Research: CDC HIV surveillance data — racial/ethnic and sexual orientation disparities in HIV rates.

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Systemic barriers prevent access to

life-saving treatments like

PrEP

Research: CDC PrEP uptake data by race/ethnicity; AIDSVu coverage gaps.

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High costs

* Lack of access to healthcare

* Stigma & discrimination

all contribute to these disparities.

Research: KFF or Commonwealth Fund report on cost/access barriers; stigma and treatment-seeking literature.

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LGBTQ+ individuals and POC often receive

disproportionately inferior treatment

and face dismissal of symptoms.

Research: Racial bias in pain treatment (Hoffman et al. 2016, PNAS); Lambda Legal/National LGBTQ Task Force reports on healthcare discrimination.

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This discrimination leads to avoidance of

crucial treatment.

Research: Medical mistrust and treatment avoidance research — connects naturally from slide 15 sources.

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This directly leads these communities towards

later diagnoses & worse health outcomes.

Research: Late HIV diagnosis rate disparities by race (CDC); cancer staging at diagnosis by race.

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The AIDS crisis emerged in the early 1980s, claiming the

lives of

over 39 million people worldwide.

Research: UNAIDS global HIV/AIDS statistics — specify the cumulative figure and data year.

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Do your part:

- * Get yourself tested
- * End the stigma behind STDs
- * Educate yourself
- * Encourage your loved ones to get tested

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- # Awareness is only effective in numbers.
- # Share this post among your community.
- # Let's work towards a better future together.

Set 3 — Rebuilding Trust

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- # Rebuilding Trust Between Marginalized
- # Communities &
- # Healthcare Providers

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- # Rebuilding trust is critical to closing the gap in medical care for marginalized communities.
- # Okay sounds cool, but...

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- # How Do We Start?

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Support Community-led Clinics

Clinics run by LGBTQ+ & POC often provide safe, affirming care.

Research: Research on community health centers and affirming care outcomes; optionally link to clinic directories (Howard Brown, Whitman-Walker, Callen-Lorde).

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Representation Matters

Patients trust providers who reflect their race, culture, & experiences.

Research: Race-concordance study — Alsan, Garrick & Graziani (2019, AER) on Black male patients and Black doctors; similar SGM provider-concordance research.

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Cultural Competency is Essential

Training doctors and nurses to

understand diverse experiences

is key to changing how

marginalized communities

receive care.

Research: Cultural/structural competency training outcomes literature (AAMC, NEJM, JAMA).

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“We do have the power, if we come together, to make

change.” -Dr. Karthik

Sivashanker

Research: Original source + year for this quote.

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Advocate for Policy Changes

We need policies that require inclusive care and

punish discriminatory practices enforced in healthcare systems.

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Community support for policy changes is necessary to ensure

inclusive healthcare for all!

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Share this to spread awareness so we can secure a future with healthcare that is:
inclusive

* affordable

* accessible

* culturally competent